

ACUTE COVID-19

Assess the patient with acute COVID-19 infection or likely infection (presumptive COVID-19)

Assess	Note
Symptoms	Manage symptoms as on symptom pages.
Chronic condition/s	- If patient has chronic condition, check that it is well controlled. Specifically ask if patient has diabetes. - If known diabetes and no HbA_{1c} result in past 3 months: take HbA_{1c} and creatinine today.
Best place for care	If known diabetes and any of: ≥ 60 years, chronic kidney disease, random fingerprick glucose > 11 with ketones present in urine or patient known with very poor glucose control (HbA_{1c} $> 10\%$) and another risk factor such as: BMI¹ > 30, hypertension, ischaemic heart disease, peripheral vascular disease, previous stroke/TIA, HIV, TB, cancer, chronic respiratory disease, discuss referral for early admission.
Diabetes screen	If not known with diabetes and any of: BMI ¹ ≥ 30 , hypertension, family history of diabetes (parent/sibling), symptoms suggestive of diabetes ² or diabetes during pregnancy, check glucose $\curvearrowright 17$.

Advise the patient with acute COVID-19 infection or likely infection (presumptive COVID-19)

- Advise patient to inform household members to use strict hygiene and prevention measures and monitor themselves for symptoms. Close contacts no longer need to quarantine or isolate, even if symptoms develop. Advise to use a mask and avoid indoor social gatherings as much as possible for at least 5 days.
- Advise the patient with known diabetes:
 - Explain that s/he is at risk of severe COVID-19. Advise to go to nearest emergency centre if s/he develops shortness of breath, weakness or high fevers/chills.
 - Advise to check glucose each morning upon waking and keep a record: if fasting glucose persistently ≥ 8 , advise to return for review of insulin doses.
- Check patient understands to monitor symptoms at home (see red box below).
- Check patient understands how to safely isolate. Refer to community-based services for follow up if available.
- Provide medical certificate for sick leave for 7 days from date that symptoms started. This may need to be extended.
- Explain that patient may discontinue isolation 7 days after date that symptoms started. If symptoms have not resolved by 7 days, advise to continue isolating until 10 days completed.

Treat the patient with acute COVID-19 infection or likely infection (presumptive COVID-19)

For fever/pain, advise to take **paracetamol** 1g 4-6 hourly (up to 4g in 24 hours) orally as needed, rather than NSAIDs⁴. If using NSAIDs⁴ for other condition/s, avoid discontinuing.

Review the patient with acute COVID-19 infection or likely infection (presumptive COVID-19)

Advise that there is no need to return to facility unless condition worsens. Advise to return for TB test if cough persists ≥ 2 weeks. Ensure correct contact details. Include a second phone number.

Advise to return urgently to health facility if:
Shortness of breath, difficulty breathing, persistent chest pain/pressure, new confusion or worsening drowsiness.

¹BMI = weight (kg) $\div$ height (m) $\div$ height (m). ²Weight loss, thirst (especially at night) or passing excessive amounts of urine often. ³Non-steroidal anti-inflammatory drugs (like ibuprofen).